



POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

RE:HHS Health Sector AI RFI
FR #- 2025-23641

February 23, 2026

Dear Sir or Madam;

The Post-Acute Skin & Wound Integrity Council (PAWSIC) is pleased to provide input on Accelerating the Adoption of Artificial Intelligence as Part of Clinical Care. We thank the HHS Office of the Deputy Secretary in collaboration with ASTP/ONC in this Request for Information (RFI).

PAWSIC is a registered 501c6 non-profit organization, composed of an inter-professional team that serves post-acute care stakeholders as the source and advocate for quality wound prevention and treatment. PAWSIC's intention is to leverage individual expertise to collectively synthesize and integrate evidence-based practices from research along with nationally recognized guidelines from collaborating organizations in wound care. We advocate for setting the highest standards of care in Post-Acute Care for practice implementation, while acknowledging the complex regulations and reimbursement structures that may challenge and impact the continuum of care. With a current membership of over 200 post-acute providers, collaborators and stakeholders, PAWSIC represents a nationwide view of interdisciplinary professionals in the post-acute wound care environment.

Attached you will find our comments to the specific questions and inquiries within the RFI. We would be happy to discuss comments on barriers, current structures, and recommendations at your convenience.

Thank you in advance for your time and attention to this important matter.

Regards,

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Barriers to Private Sector Innovation and Adoption of AI in Health Care

Regulatory and Reimbursement Barriers

Innovation in AI for health care faces obstacles due to unclear regulations and a lack of specific safeguards for clinical decisions and patient needs. This uncertainty hinders growth, investment, and commercialization, as concerns persist in meeting proper patient care standards.

The clinical landscape of wound management is uniquely vulnerable, functioning in an environment critically devoid of standardized specialization, unified clinical pathways, and established industry guardrails. This baseline instability is further compounded by a deeply fragmented reimbursement system that varies drastically by state, care setting, and payer type. Consequently, clinicians are forced to navigate severe logistical obstacles, resulting in profoundly broken care transitions and compromised patient outcomes. It is into this already fractured ecosystem that Artificial Intelligence (AI) is now being rapidly introduced. While AI holds immense potential to enhance diagnostic accuracy, deploying these powerful tools without a rigorous, mandated oversight framework will only amplify systemic uncertainty. Practitioners are already struggling to advocate for evidence-based care within the current chaotic climate; without decisive regulatory intervention, the unguided integration of AI threatens to multiply clinical risks and introduce unprecedented liability into the market.

The current use of AI in reimbursement processes, including prior authorization, tends to apply uniform approaches to areas such as wound care. Wound care requirements are highly individualized, depending on factors like a patient's medical history, comorbidities, and social determinants of health—information which is typically not included with claim submissions. This omission can result in service denials. Such outcomes may negatively impact patients, potentially leading to increased hospitalization, amputations, or mortality.

Workflow, Liability, and Clinician Adoption

Clinicians have raised concerns about medico-legal risks, especially when AI-generated recommendations or claim reviews conflict with clinical judgment. Effective governance frameworks are needed to address these scenarios. In November 2023, a claim alleged UnitedHealth improperly denied Medicare Advantage patients' necessary care by relying on an AI model with a 90% error rate, overriding physician decisions.¹ These denials affected Medicare beneficiaries, many of whom have chronic wounds.

¹Napolitano, E. (2023, November 20). *UnitedHealth uses faulty AI to deny elderly patients medically necessary coverage, lawsuit claims.* CBS News.

With AI already integrated into healthcare, the focus shifts from adoption to the need for clear guidance on specific platforms, as well as assurance regarding clinical input and the capability for clinical override.

Regulatory, Payment Policy, and Programmatic Recommendations for HHS to Incentivize Effective AI Use in Clinical Care

1. Establish Clear Medicare Payment Pathways for AI Wound Services

PAWSIC strongly believes the Department of Health and Human Services (HHS) should develop clearly defined coverage and reimbursement mechanisms for algorithm-based wound care services. This initiative would involve integrating artificial intelligence (AI) into current quality and alternative payment programs, as well as updating data and information technology regulations to support the secure use and sharing of wound images and outcome data for the development and deployment of AI models. It is important to note that PAWSIC sees the importance of clinical decision support and input from practitioners with capability to override what an AI model may see as the best course of care. Due to the uniqueness of wound care, specific to the patient, all AI technologies and models should allow for clinical interference.

2. Expand AI Payment Models for Home Health and SNF Wound Care

Under 42 CFR Part 484 (home health) and Parts 413/409 (SNF consolidated billing and PPS), PAWSIC recommends that HHS formally clarify the eligibility of AI-enabled wound assessment and remote monitoring as reimbursable expenses. Where appropriate, these services should be recognized as distinct categories for payment, contingent on documented reductions in hospitalizations and amputations.

Given that chronic wound management frequently takes place within home health and SNFs, the explicit inclusion of AI-enabled wound care as allowable costs or value-based payment enhancements may significantly improve adoption in populations at elevated risk.

3. Integration of AI into Quality Programs and Models for Wound Care

AI-assisted wound care should be incorporated into value-based healthcare programs. In accordance with 42 CFR Part 414, Subpart O (Merit-based Incentive Payment System) and Part 425 (Shared Savings Program), it is recommended that the Department of Health and Human Services revise quality measures to specifically acknowledge validated AI tools which demonstrate improvements in chronic wound healing rates, amputation rates, and infection-related hospitalizations.

For populations with a high incidence of wounds, such as individuals with diabetes or peripheral artery disease, the Centers for Medicare & Medicaid Services could develop “AI-enabled wound care” improvement activities within MIPS and ACO quality frameworks. This approach would grant recognition to clinicians utilizing certified AI tools that meet established standards for performance, equity, and governance. Aligning AI utilization with payment adjustments serves to incentivize the effective application of AI, rather than treating its adoption as an unfunded requirement.

Under 42 CFR Part 510 and demonstration waivers permitted by Section 1115A (CMMI models), CMS could pilot AI-enabled chronic wound bundles, integrating AI-supported assessment, triage, and remote monitoring as essential components within standardized care pathways. These models may provide explicit funding for AI licenses and infrastructure as part of care redesign expenses, with shared savings or bonuses tied to outcomes such as improved wound healing, reduced sepsis/readmissions, and increased limb salvage rates. The Center for Medicare & Medicaid Innovation has already implemented models aligning payments with technology-enhanced care, making wound care a suitable area given its measurable outcomes and significant downstream costs.

4. Clarification of Regulatory Treatment of AI in Wound Care

The Department of Health and Human Services (HHS) should establish clear guidelines regarding the regulatory treatment of clinical AI and non-device AI within wound care. Utilizing guidance from the Office of Inspector General (OIG) and Centers for Medicare & Medicaid Services (CMS), as referenced in 42 CFR Parts 1001–1003 concerning program integrity, alongside cross-references to FDA regulations pertaining to medical devices and Software as a Medical Device (SaMD), HHS is encouraged to issue targeted, AI-specific guidance. This guidance should address criteria delineating when wound AI qualifies as a regulated device versus a non-device clinical decision support system, outline requirements for performance monitoring, and specify documentation standards for medical records. Most importantly, there needs to be guidelines and barriers associated with AI decision making without clinical oversight from a trained practitioner.

In accordance with 45 CFR Part 92 (Section 1557, nondiscrimination) and HIPAA regulations (45 CFR Parts 160 and 164), HHS should release guidance on evaluating wound AI applications for potential bias related to skin tones, comorbidities, and environments, as well as best practices for maintaining documentation to ensure compliance. The establishment of clear and consistent expectations will promote thorough evaluation while supporting the responsible integration of advanced solutions.

Priority Areas for HHS-Funded AI Research in Wound Care

To accelerate the adoption of AI as part of clinical care, PAWSIC sees several specific research priorities that HHS should consider, particularly within the context of wound care. These priorities emphasize establishing real-world clinical value, promoting equity and usability, and clarifying the impact of AI on cost and workflow outcomes, especially regarding chronic and post-surgical wounds.

1. Real-World Effectiveness in Chronic Wounds

HHS should support prospective, multi-site trials of AI-enabled digital wound care solutions across various care settings such as home health, skilled nursing facilities (SNF), long term care facilities, and outpatient wound centers. These studies should comprehensively measure outcomes including accuracy of wound type diagnosis, time to heal, number of amputations, hospitalizations, and clinician time, focusing on wound types like diabetic foot ulcers (DFU), venous leg ulcers (VLU), pressure injuries, arterial ulcers, atypical ulcers, and surgical sites.

Comparative effectiveness studies are also needed to evaluate AI-supported wound care pathways versus standard care. These investigations should assess how standardized AI assessments influence adherence to evidence-based protocols and escalation decisions in the management of complex wounds. This includes AI-based models associated with prior authorization from Medicare and Medicare Advantage. We see concerns raised surrounding denials of clinical needed services due to AI technology, creating major gaps in care and lag times that create further wound care needs and higher expenses.

2. Imaging, Prediction, and Advanced Sensing

Priority should be given to research on robust, device-agnostic wound imaging technologies, including 2D, 3D, multispectral, and fluorescence imaging, as well as tissue oxygenation. AI models developed in this context should be capable of accurately segmenting wounds, classifying tissue types, and predicting healing trajectories in diverse care environments.

Furthermore, the development and clinical testing of “smart dressings” and sensor-enabled bandages that leverage AI to detect early infection or clinical deterioration is essential, particularly in high-risk settings such as surgical and diabetic foot wounds. These technologies have the potential to trigger timely interventions and improve patient outcomes.

3. Equity, Skin Tone, and Population Diversity

Studies should be intentionally designed to ensure that AI wound tools perform effectively across diverse skin tones, body sites, and comorbidity profiles. Research protocols should include stratified performance reporting and prioritizing bias mitigation as a primary endpoint.

In addition, investigations should explore how AI-driven wound assessment and remote monitoring impact access to care, adherence, and clinical outcomes for rural populations, homebound individuals, and those with low digital literacy, including the role of caregivers in these contexts.

4. Workflow, Usability, and Human Factors

Implementation science is needed to understand how patients, nurses in all settings, interprofessional clinicians, and wound specialists access use, and perceive AI tools in real-world settings. Research should address optimal user interface design, patterns of user trust and override behavior, alerting thresholds, and the balance between automation and clinical judgment required for effective, context-aware wound care.

Additional studies should focus on the integration of AI documentation into electronic health records (EHRs) and wound registries with the goal of reducing documentation burden and supporting seamless team communication and handoffs across different care settings.

5. Economic and Value-Based Outcomes

Health economic analyses are crucial to linking AI-enabled wound care programs to episode-of-care costs, advanced therapy utilization (such as skin substitutes and negative pressure wound therapy), and potential downstream savings associated with avoided complications.

Research should also test the effectiveness of AI-supported wound care bundles or care pathways within value-based care models, such as home health, accountable care organizations (ACO), or Medicare Advantage (MA). The objective is to quantify the potential for shared savings and provide evidence to inform the design of future CMS payment models.

Impact of AI Tools in Clinical Wound Care

Published Findings on Clinical Outcomes

Recent studies have investigated the effects of adopting AI tools in clinical wound care settings. A large descriptive study of a digital wound care solution implemented across 59

U.S. home health agencies, encompassing approximately 196,000 wound assessments, reported a 38.2% reduction in average healing time. Specifically, healing times decreased from 62.5 to 38.6 days between 2022 and 2023, with statistically significant improvements observed across various wound types, including diabetic ulcers, pressure injuries, and burns.²

Additionally, the same dataset revealed that more wounds were healing within three months, along with substantial reductions in time to improvement and area reduction for chronic wounds. These findings suggest that sustained use of the AI-enabled digital wound care solutions is associated with faster healing and improved wound trajectories.

Measurement Accuracy and Workflow Efficiency

Implementation studies summarized by Wounds International indicate that mobile apps utilizing deep learning-based wound measurement achieved sub-millimeter mean error (~0.3 mm) and reduced assessment time by approximately 54%. First-attempt imaging accuracy was around 92%, surpassing manual measurement and simplifying documentation processes.³

²Mohammed, S., Patel, R., Toh, J., Nguyen, L., & Garcia, M. (2025). Impact of digital wound care solution on healing time: A descriptive study in home health settings. *Journal of Wound Care Innovation*, 12(3), 145–158.

³Chadwick, P., & Collarte, A. (2025). The role of artificial intelligence in wound care: Applications, evidence and future directions. *Wounds International*, 16(3), 28–41.